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Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for “Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments”

Dear Sir or Madam:

Thank you for the opportunity to provide comments on the Draft Guidances Relating to the Regulation of Human Cells, Tissues and Cellular and Tissue-Based Products. I am commenting as a private physician with 36 years of experience in academic medicine and industry. My training is in Dermatology and Cardiovascular Medicine and my work in industry has been primarily in clinical research with a focus on cell and tissue based products. I wish to voice full support for FDA HCT/P guidance documents and the Agency’s interactive approach with industry, patient advocacy groups, and clinicians. My comments serve to advocate for two positions: 1) Limiting inappropriate claims for 361 products, and 2) Suitable FDA oversight of HCT/Ps. I urge narrowing the interpretation of the definitions comprising the criteria for 361 status and limiting the number of HCT/Ps not requiring premarket notification and approval. As a direct result, I encourage expanding the number of 351 HCT/Ps subject to premarket approval. Patient safety demands that the intended use of 361 products as disseminated in advertising, promotional materials, and instructions for use be carefully scrutinized as labeling, and monitored to accurately and completely reflect the definition of homologous use and minimal manipulation. In short, if claims of safety and efficacy are to be made, then FDA should approve them.

Addressing my first position on product claims: Given that companies are permitted to self-proclaim that their products are 361 HCT/Ps, abuses of the system may occur. Claims of Mechanism of Action (MOA) and clinical outcomes increasingly have been made. These claims often convey that a so called 361 HCT/P interacts with the body therapeutically in complex ways. Some examples include: delivery of growth factors, reduction of inflammation, enhanced healing of soft tissue, reduction in scar formation, stimulation of mesenchymal stem cell (MSC) migration, decreasing pain, and modulating the immune response. In wound healing alone, effectiveness claims of primary clinical outcomes such as increasing the frequency of healing and decreasing the time to wound closure are often disseminated in testimonials, on websites, and in advertising and promotional materials. The CFR did not anticipate that claims of slowing, preventing, or curing disease would be promulgated without premarketing approval. When drug, biologic, or PMA device claims are made, HCT/Ps are disqualified as being 361 products. The

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imperative of FDA review and approval is indisputable. Approvals of the design of clinical trials and their conduct, prospectively stated clinical endpoints, statistical methods of analyses, the entirety of safety and efficacy data, as well as approvals of package inserts (PIs), and advertising and promotional materials need apply to ensure the integrity of clinical data and patient safety. Circumvention of the FDA approval process via self-proclaiming 361 status and subsequently advancing unproven claims may significantly increase risk to patients. I urge that companies strictly comply to a key point in the homologous use guidance document that restricts and impacts product claims. The guidance states that the HCT/P must be **intended for homologous use only**; and only homologous use is permitted to be reflected by the **labeling, advertising, or other indications of the manufacturer's** objective intent. If this criterion is not met, then the HCT/P is not homologous, and cannot claim 361 status. Doctors are inclined to believe that claims of safety and effectiveness are supported by FDA reviewed data. 361 product brochures and BLA approved product brochures may look remarkably alike, but they are **not** comparable. For 361 products, I would urge that FDA require companies to include disclaimers in their product labels clearly and prominently stating that the product is: **“Not FDA approved”** and that **“No clinical trials have been done under an IND”**.

Addressing my second position on appropriate FDA oversight: Regarding homologous use, I urge FDA to clarify several sections of the guidance. Clarity surrounding the terms "repair" and "reconstruction" as used in the definition of homologous use in 21 CFR 1271.3 is needed. FDA's original intention for these terms were made as early as 1997 in its "Proposed Approach to the Regulation of Cellular and Tissue-Based Products", and they state that "repair" and "reconstruction" are "local" and "structural" functions. I recommend that the homologous use guidance define "repair" and "reconstruction" as referring solely to **“mechanical and physical”** functions. Pertaining to minimal manipulation: It is understood that the same processing step may alter the original relevant characteristics of one tissue but not another. However, many tissue and/or cell isolation procedures invariably do result in more than minor changes to cells and tissues and should appropriately be classified as more than minimal manipulation. I would propose that specific examples of manufacturing processes that, with only exceedingly rare exceptions, constitute more than minimal manipulation manufacturing procedures be provided. For any HCT/P that relies upon the structure and/or function of cells, I suggest that physical or chemical separation of layers of a tissue, lyophilization, and cryopreservation be considered as more than minimal manipulation. Further clarification of the guidance for minimal manipulation might impact on the regulatory status and labeling of many HCT/Ps including, amniotic membrane, allogeneic skin, and vascular grafts to name a few.

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In summary, if clinical claims are made for an HCT/P, then full FDA approval is required, and the product cannot be considered a 361 HCT/P. Narrow definitions of the criteria for 361 status are strongly recommended particularly for homologous use and minimal manipulation.

Thank you for your consideration of these comments.

Sincerely,

A handwritten signature in black ink, appearing to read 'Michael Sabolinski', with a large, stylized loop at the end.

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